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Document control

Field	Entry
Title	Intended Use Statement for Adaptive Triage
Reference	PT-2.1-Intended Use Statement (reference to be confirmed by NHSE Regulatory Affairs)
Status	Draft
Version	0.5
Author	Martin Hodgson
Owner	[TO BE COMPLETED]
Version issue date	TBC
Device	Probabilistic Triage: an AI-enabled probabilistic triage capability
Classification	**Class I**, Rule 12, Annex IX to Directive 93/42/EEC as given effect by UK MDR 2002
Conformity route	Self-declaration under Annex VII. No Approved Body.
Software safety class	**Class C**, IEC 62304
Applicable framework	UK MDR 2002 (SI 2002/618, as amended), Part II, regulation 7(1)
Territorial scope	Great Britain

Revision history

Version	Date	Summary of changes	Revised by
0.1	TBC	Initial draft.	Martin Hodgson

Reviewers

Reviewer name	Title / responsibility	Date	Version

Approved by

Name	Title	Date	Version

Related documents

Document ID	Title
PT-2.1-A	Platform Functionality Set Register and Intended Purpose by Set
NHSP-CIR	Classification and Intended Purpose Rubric v0.4
CP-2.1	Core Pathways Intended Use Statement
CP-2.2	Core Pathways Indications for Use
CP-2.3	Core Pathways Clinical Claims and Benefits
CP-2.4	Core Pathways Target Patient Population
CP-2.5	Core Pathways Contraindications
CP-2.6	Core Pathways Intended Users
CP-2.7	Core Pathways Intended Use Environment
CP-2.8	Core Pathways Foreseeable Misuse
NHSP-10.3	Clinical Evaluation Plan and Clinical Evaluation Report
NHSP-10.4	Post-Market Surveillance Plan
MD-QMS-SOP-D001	Software Design Control
MD-QMS-SOP-D002	Medical Device Risk Management
MD-QMS-SOP-D007	Software Change Management
MD-QMS-SOP-D011	Labelling and Information for Users
MD-QMS-SOP-D017	Usability Engineering
MD-QMS-WI-D002	Software Safety Classification
[ref]	Clinical Risk Management Plan and Clinical Safety Case Report (DCB0129)
[ref]	Risk Management File (BS EN ISO 14971)
[ref]	Usability engineering file (BS EN 62366-1)
[ref]	Interface specification, Probabilistic Triage service
[ref]	NHS Pathways host system accreditation requirements

2.1 Intended Use Statement

2.1.1 Scope

Purpose of this document

This document defines the intended purpose of Adaptive Triage as declared for conformity assessment, registration and clinical evaluation.

It is the design input for risk analysis under BS EN ISO 14971 and MD-QMS-SOP-D002, for usability engineering under BS EN 62366-1 and MD-QMS-SOP-D017, for clinical evaluation under NHSP-10.3, and for the clinical safety case under DCB0129. Its content follows BS EN 62366-1 clause 5.1, BS EN ISO 14971 clause 5.2 and BS EN 82304-1 clause 4.1a.

The device

The device is the probabilistic triage assessment service. It comprises five components:

Component	Function
Symptom assessment and disposition generation	Evaluates reported symptoms, demographic attributes and reported history against governed NHS Pathways content and returns a triage disposition.
Associated care parameters and safety-netting content	Returns the timeframe, the general level of clinical skill required, SNOMED CT concepts describing the assessment and its outcome, and first-aid, immediate-action, safety-netting and worsening advice attached to the disposition.
Adaptive question selection and evidence accumulation	Selects the next question from the evidence accumulated so far and determines when sufficient evidence has been gathered. Authors the question text and permitted answer options.
Disposition confidence value	Returns a numeric or banded value expressing the device's confidence in the disposition it has returned.
Clinical administration and content governance interface	Allows authorised clinical staff to manage content versions, thresholds, disposition mappings and review of model behaviour.

These correspond to functionality sets FS-03 to FS-07 in PT-2.1-A. The device is deployed as a service. Approved host products call it through a defined interface, render its content and act on its output.

Naming

Adaptive Triage: an AI-enabled probabilistic triage capability.

2.1.2 Definitions of terms and acronyms

Term or acronym	Definition
Adaptive economy	The reduction in the number of questions asked, achieved by selecting each question for the uncertainty it resolves and by stopping once the stopping threshold is met
Annex VII, Annex IX, Annex X	Annexes of Council Directive 93/42/EEC as given effect in Great Britain by the UK MDR 2002: Annex VII is the EC declaration of conformity route used for Class I devices, Annex IX the classification criteria, Annex X the clinical evaluation requirements. Essential Requirement 3 is the third of the Essential Requirements at Annex I.
CAS	Clinical Assessment Service
Contraindication	A situation, disease or condition specifically excluded from the intended use
CSO	Clinical Safety Officer, as defined in DCB0129
Determined population	The population that receives care shaped by an output, whether or not it operates the device. Distinguished from the direct user.
Direct user	Whoever operates the device or reads its output. May be a system.
Disposition	A category drawn from the NHS Pathways disposition set, indicating the type of care recommended or that self-care is appropriate
Disposition set	The controlled set of disposition categories published as part of the NHS Pathways clinical content release. No member of the set names a clinical condition. The set applicable to a given release is identified by that release, and the enumeration is annexed to this statement.

DoS	Directory of Services
Emergency screen	The part of the assessment that identifies presentations potentially requiring an emergency response. It runs before adaptive economy is applied and is not subject to early stopping.
Functionality set	A group of software functions on the platform sharing a single declared intended purpose and therefore a single classification determination. The unit at which intended purpose is declared across the platform. Sets FS-01 and FS-02, the non-clinical platform services and the data conduit, are not medical devices and are outside this statement. FS-03 to FS-07 are the five components of the declared device. The full register is PT-2.1-A.
MDP	Medical Device Programme, the NHS England body that owns qualification and classification triage, regulatory strategy, MHRA registration, and serious adverse incident reporting
Host product	An approved software product that calls the device, renders its content to a user and acts on its output. Outside the device boundary.
Indications for use	The clinical condition to be diagnosed, prevented, monitored, treated, alleviated, compensated for, replaced, modified or controlled by the device. Distinguished from intended purpose, which describes the general use or function for which the device is intended.
Intended purpose	The use for which a device is intended according to the data supplied by the manufacturer on the label, in the instructions for use or in promotional or sales materials or statements, and as specified by the manufacturer in the clinical evaluation. Intended use may be substituted for intended purpose in all instances.
Lay person	An individual who does not have formal education in a relevant field of healthcare or medical discipline
Negative indication	A statement, appearing on the labelling and in the user-facing product, that a named function or output is outside the intended purpose. Has regulatory effect only where it is user-facing.
NHSP-CIR	Classification and Intended Purpose Rubric, the controlled method by which every functionality set on the platform is qualified, classified and gatekept. It defines the determination sheet, the separation test and the contamination controls, and it is the method under which any change to this statement is reassessed.
Provider Licence	The licence under which providers of NHS 111 and NHS 999 services operate the product
Licence to Embed	The licence under which host system suppliers embed or call the product
RA	Regulatory Affairs, NHS England
SaMD	Software as a Medical Device
Single contact episode	One continuous assessment initiated by one contact from or on behalf of one individual, ending when a disposition is returned or the assessment is abandoned. A subsequent contact, including a re-contact after a disposition and a transfer to another service, begins a new episode with no carried-over evidence.
Stopping threshold	The level of certainty about the disposition at which the device stops asking questions. A safety-critical parameter governing undertriage risk, held in the Risk Management File.
SNOMED CT	Systematised Nomenclature of Medicine Clinical Terms
UK MDR 2002	The Medical Devices Regulations 2002 (SI 2002/618), as amended

2.1.3 Intended Purpose

Adaptive Triage is a Software as a Medical Device that provides a structured, symptom-based clinical assessment to support triage within NHS urgent and emergency care services. The device evaluates patient-reported symptoms, demographic attributes and reported clinical history against clinically authored and governed NHS Pathways content, using probabilistic inference to guide the order of questioning and to determine when sufficient evidence has accumulated. Based on the information entered during a single contact episode, the device generates a triage disposition drawn from the NHS Pathways disposition set, the timeframe within which care should be accessed, the general level of clinical skill required for onward care, associated safety-netting and immediate-action content, and a value expressing the device's confidence in that disposition.

The device is intended for use within NHS urgent and emergency care services, including NHS 111, NHS 999 and Clinical Assessment Services, and within approved digital self-service channels. It is deployed as a service and is accessed by approved host products through a defined interface. The determined populations are members of the public using approved self-service channels and individuals assessed by trained non-clinical health advisors in assisted channels. The device also provides a clinical administration interface through which authorised clinical staff review content, thresholds and model behaviour.

The device provides information on the route to care and the urgency of access to it. It does not provide a diagnosis, does not generate or provide any list of candidate clinical conditions or any likelihood attached to a clinical condition, does not initiate or manage treatment, does not allocate a named service provider or appointment, and does not commit any clinical action on its own behalf.

Sections 2.2 to 2.8 below set out the indications, claims, population, contraindications, users, environment and foreseeable misuse that give this statement its full content. Section 2.1.4 gives only the short structural summary that CP-2.1 itself carries; the detailed technical breakdown is at 2.2.

2.1.4 Structure and Function of the Device

NHS Pathways triage works by ruling out, hierarchically: the most life-threatening problems first, then questions specific to the individual's main problem, with symptom-specific pathways themselves ordered by decreasing need for urgent attention. Probabilistic Triage retains that clinical logic and changes how the path through it is chosen. The device is a probabilistic triage assessment engine governed by a Bayesian network, reasoning over dispositions rather than over clinical conditions, that evaluates evidence across multiple presenting symptoms at once and selects each next question for the reduction in uncertainty it produces about the disposition.

The full technical breakdown, including the clinical process and workflow role, inputs, outputs, performance characteristics, the software architecture summary and the accessories and extraction boundary position, is at 2.2 Indications for Use.

2.1.5 How it is Supplied

The device is supplied as a service. Approved host products submit accumulated evidence through a defined interface and receive, in response, the next question and permitted answers, and subsequently the disposition and its associated parameters. The interface specification forms part of the technical documentation, because it defines what the device receives and what it returns.

Providers of NHS 111 and NHS 999 services using the device as their triage tool operate within the product's Provider Licence. Host system suppliers embedding or calling the device operate within the product's Licence to Embed. Host system suppliers are treated as critical suppliers within the NHS England quality management system.

2.2 Indications for Use

2.2.1 Medical Purpose and Clinical Process Role

Medical purpose. The device provides structured, adaptive triage and guidance to support trained non-clinical health advisors, and members of the public in approved self-service channels, in assessing reported symptoms and directing individuals to appropriate care pathways or self-care options across NHS urgent and emergency care services. Outputs support decisions on care routing and the urgency of access to services. The triage approach prioritises the identification of potential urgent or emergency needs, followed by direction to appropriate care pathways or self-care options.

Point of use. The device is used at the initial point of contact, whether by telephone in assisted channels or through a digital self-service channel, to support structured triage of individuals presenting with any symptom or health concern. In assisted channels it guides a trained non-clinical health advisor; in self-service channels it guides the individual directly. Both are governed by the same clinically authored decision logic.

Outputs generated. Based on user-entered information, the device generates triage outputs that provide guidance on appropriate care pathways, including recommended service type, required skill set and timeframe for access, or self-care where appropriate. Full detail is at 2.2.4 Outputs.

Boundary of action. The care-routing decisions and operational protocols that act on these outputs are performed within NHS service workflows and sit outside the device boundary. The device functions as an entry-point triage system within those workflows, including NHS 111, NHS 999 call handling and Clinical Assessment Services. Outputs may inform subsequent decision-making processes but are considered alongside additional operational and clinical inputs where applicable, and mapping of a disposition to an available service is performed downstream by the Directory of Services or by NHS 999 configuration mapping, outside the device.

2.2.2 Clinical Condition and Indication

Condition or indication. Individuals presenting with any symptom, health concern or request for health-related advice requiring remote triage and assessment. The device is not limited to specific clinical conditions and supports initial assessment across all body systems, age groups and levels of acuity, including acute, non-acute and chronic presentations.

ICD-10 and SNOMED CT codes. No ICD-10 restriction applies. The device operates across all symptom presentations without restriction to a specific diagnostic code. It emits SNOMED CT concepts describing the symptoms reported, the assessment performed and the disposition reached; no concept it emits names a clinical condition as a candidate diagnosis.

Stage or severity restriction. None. The device is designed to assess presentations ranging from immediately life-threatening conditions to lower-acuity and non-urgent care needs. The device applies clinically authored decision logic to identify potential life-threatening conditions and assign appropriate triage dispositions.

Contraindications and exclusions. Summarised here and stated in full at 2.5: use for a purpose other than the declared self-service and assisted channels; use by personnel who have not completed mandatory NHS Pathways training in channels where that training is a declared risk control; use outside of defined NHS service workflows and operational environments for which the device is configured.

2.2.3 Inputs

The device operates exclusively on structured inputs received through its interface within a single contact episode.

Input	Type	Source	Validity requirements
Demographic attributes: age, sex or gender	Categorical	Host product, from user entry, Patient Demographic Service match, or advisor selection where unknown or unverified	Selection from predefined categorical values. Mandatory before assessment begins. Determines routing to an age and gender appropriate assessment.
Presenting symptom or chief	Structured selection,	Host product. Where the host derives it from a free-	Sufficient to initiate a symptom-based assessment. The device

complaint	expressed as a clinical concept	text description, the derivation is outside the device boundary.	assumes the concept received is the concept the individual intends. That assumption is a stated risk control, addressed at 2.2.7.
Symptom responses	Structured: binary, categorical, severity-ordered	Host product, captured during the assessment	Constrained to the answer options the device supplies. Sequential flow enforced. Mandatory completion of key questions before a disposition is reached.
Reported clinical history	Structured, categorical	Host product, reported by the individual during the contact	Reported, not record-derived.
Patient-reported physiological values, where a pathway accepts one	Structured numeric	Reported by the individual, having read an instrument themselves	See below.
Clinical knowledge base	Structured clinical data, graph-based relationships, taxonomy	NHS Pathways datasets mapped to SNOMED CT through a clinically validated mapping pipeline, governed through the clinical administration interface	Clinician-reviewed and signed-off concept mappings. Version-controlled and traceable to source pathway releases.

Patient-reported physiological values. Accepting a temperature, heart rate or oxygen saturation reading that the individual has read from an instrument and entered as a structured response does not engage the Rule 10 monitoring limb: the device receives a reported observation, it does not capture a signal. The obligation this creates sits in the risk file rather than in the classification. An instrument that may not be a validated medical device is contributing to an acuity decision, and pulse oximetry is known to over-read in individuals with darker skin pigmentation, which bears directly on the model's subgroup performance. Both are recorded as hazards.

Unstructured input. Free-text or contextual clinical history may be captured by the host product for record-keeping, and may be used by the host to narrow the list of pathways offered to a user. It does not enter the device's decision logic. All inputs to the decision logic are structured and are selected by the user during the live interaction.

2.2.4 Outputs

Output	Content	Recipient
Triage disposition	A category from the NHS Pathways disposition set indicating the type of care recommended, or that self-care is appropriate	Host product, rendered to the user
Timeframe	The period within which care should be accessed	Host product
Skill set	The general level of clinical skill required for onward care	Host product
SNOMED CT concepts	Concepts describing the symptoms reported, the assessment performed and the disposition reached. No concept names a candidate diagnosis.	Host product, clinical record
Safety-netting and immediate-action content	Fixed content attached to the disposition or pathway	Host product, rendered to the user

Disposition confidence value	A numeric or banded value expressing confidence in the disposition. Not a probability that any condition is present, and not a measure of severity.	Host product, rendered to the user
Next question and permitted answers	Question text and answer options, authored by the device	Host product, rendered verbatim to the user

Ambulance response standards are nationally defined. Dispositions to other care settings are applied within provider-specific service configurations. Mapping of the disposition to an available service is performed by the Directory of Services and associated provider processes, outside the device.

Outputs are acted upon within defined operational protocols: in assisted channels by trained non-clinical health advisors, in self-service channels by the individual. Routine clinical review of each output is not required by the device before the output is used. That is a stated characteristic of the intended purpose, not an omission. Escalation to a supervising clinician is available where service escalation criteria are met, and those criteria sit outside the device.

The device does not produce, and the trial configuration does not contain, any output naming, ranking or attaching a likelihood to a clinical condition.

2.2.5 Performance Characteristics

No quantitative clinical performance claim is made in this document. Performance is defined as the device's ability to provide a consistent, structured and adaptive triage assessment based on probabilistic evaluation of the information reported, in accordance with its intended purpose, and is evidenced against the acceptance criteria at 2.3.2 in NHSP-10.3.

Two consequences of the declared intended purpose apply. First, CB-1 at 2.3.2 is a performance claim and carries Essential Requirement 3 and Annex X in full. Second, NHS Pathways content was developed and validated for advisor-led telephone triage, so performance in lay self-directed use is evidenced separately. An equivalence argument does not carry a change of use context on its own.

Because the device applies probabilistic inference, the technical documentation additionally carries model validation records, training data provenance, performance analysis across demographic subgroups, and calibration evidence for the disposition confidence value.

Operational service metrics, including call handling time, assessment duration, service availability and downstream care outcomes, are influenced by service configuration, provider workflow and patient behaviour and are outside the scope of device performance. Performance is monitored under NHSP-10.4.

2.2.6 Software Architecture Summary

Software type. The device is a probabilistic triage assessment engine governed by a Bayesian network. Clinically curated NHS Pathways knowledge, mapped to standardised clinical concepts, is represented as a probabilistic model over the relationships between reported symptoms, risk factors and triage outcomes. During an assessment, each structured response is accumulated as evidence, the distribution over candidate dispositions is updated, and the next question is selected accordingly. Questioning stops when the stopping threshold is met.

The model reasons over dispositions. It contains no latent variable, node or intermediate quantity representing a clinical condition. That is a model-architecture constraint and is verified against the trained artefact rather than against the output schema.

Presentations requiring an emergency response are screened before adaptive economy is applied. Early stopping is a safety-critical behaviour, not an efficiency feature: the stopping threshold governs undertriage risk directly, and it is held in the Risk Management File with a defined and clinically assured value. The hazard reference for that value is open and is recorded at section 2.12.

The internal reasoning method is not a classification criterion under Annex IX. The device is Class I despite being probabilistic, and it would be Class IIa if it named conditions, however it reasoned.

Change management. Characterised using the change management fields of IMDRF/SaMD WG/N81:

- **Degree of learning and change autonomy:** externally controlled, manufacturer-driven change. The device does not adapt during use, and no parameter changes as a result of an individual assessment.
- **Domain of change implementation:** national.
- **Update infrastructure:** service, with host products calling the current released version through the interface.

Updates to the knowledge base, conditional probability tables and model parameters are made offline, validated and released under the quality management system, and assessed for significance. Each is a design change under ISO 13485 clause 7.3.9 and routes through MD-QMS-SOP-D007. Changes touching design or intended use return to MD-QMS-SOP-D001 Gateway 0 via a Change Requirements Specification. A stated refresh cadence of monthly or quarterly is a substantial volume of design changes, and the change control process has to be able to absorb it.

The device does not operate under a Predetermined Change Control Plan. The PCCP pathway proposed in the draft 2026 Regulations is not in force, and a PCCP requires approval by a UK approved body, which a Class I self-declared device does not engage. Model updates are therefore handled as ordinary design changes under the quality management system, and the refresh cadence has to be affordable on that basis. If the device later moves to Class IIa, a PCCP becomes available and the change control position is restated at that point.

A new output type emerging from a retrained model is the failure mode a retraining process is least likely to look for. Every model release is therefore checked against the boundary conditions at section 2.11 before it is authorised.

2.2.7 Accessories, Host Products and the Extraction Boundary

Approved, wherever it appears in this statement, means accredited against the NHS Pathways host system accreditation requirements and admitted to the product's Licence to Embed or Provider Licence. It is a risk control and not a descriptive adjective.

The device requires deployment via an approved host product to fulfil its intended purpose. The host provides the user interface, telephony integration and data handling environment. No hardware accessory or companion physical device is required.

The regulatory status of the host product must be stated, and one of three positions must be chosen: accessory within the meaning of the UK MDR 2002, part of the IT environment in which the device operates, or component of the finished device. The position determines whether the host carries manufacturer obligations, whether a system-level conformity position is required, and where assurance for newly onboarded hosts sits. The accreditation framework may also be subject to a separate quality agreement between NHS England and the host system supplier. This determination is open and is a prerequisite to approval of this statement (see 2.12).

The extraction boundary. Free-text symptom capture, in which an individual describes a problem in their own words and the description is mapped onto structured clinical concepts, sits with the patient-facing product rather than with the device. The boundary is defensible and it leaves a failure mode the device cannot own: if the extraction returns the wrong concepts, the device reasons correctly on the wrong evidence and produces a well-founded disposition for a presentation the individual does not have. Nothing inside the device can detect that. Three things follow, and they are prerequisites rather than actions. The extraction component's own regulatory status is determined by its owner, since mapping a clinical description onto clinical concepts is itself a candidate medical device function. The interface specification states what the device assumes about the concepts it receives, because that assumption is the risk control. And the residual risk is carried explicitly: by the extraction component, by a confirmation step shown to the individual before the assessment proceeds, or by a recorded acceptance in the device's risk file with the reasoning stated.

2.3 Clinical Claims and Benefits

2.3.1 Comparative Claims

No comparative medical, usability or technical claims are made about the safety or performance of the device against an equivalent device. This mirrors the position CP-2.3 records for Core Pathways, but it is not carried across as a standing fact: Core Pathways records that a literature search across UK and European sources found no known equivalent SaMD for its intended use. No equivalent search has yet been completed for the addition of probabilistic, adaptive questioning and a disposition confidence value on top of that clinical content. This is recorded as an open item at 2.12, to be closed by the literature search conducted for the Clinical Evaluation Report (NHSP-10.3) before this section is finalised.

Pending that search, no equivalence claim is made and none should be inferred from the absence of a known comparator. Usability: NHS Pathways has legacy status, predating BS EN 62366-1, and a summative usability study has been completed for Core Pathways; the corresponding evaluation for the probabilistic components and the self-service channel is tracked in the usability engineering file rather than asserted here.

2.3.2 Claimed Clinical Benefits

Neither the UK MDR 2002 nor Directive 93/42/EEC defines clinical benefit. The definition used is that in MEDDEV 2.7/1 Rev 4, the clinical evaluation guidance for this framework: the positive impact of a device on the health of an individual, expressed in terms of meaningful, measurable, patient-relevant clinical outcomes, including outcomes related to diagnosis, or a positive impact on patient management. Each claimed benefit below is therefore stated as a measurable outcome with an acceptance criterion, and effectiveness against it is demonstrated by documented clinical evidence under Annex X of Directive 93/42/EEC.

ID	Claimed benefit	Effectiveness target	Where evidenced
CB-1	Safe identification of presentations requiring an emergency or urgent response	Undertriage rate at or below the acceptance criterion for time-critical presentations, set in the Clinical Evaluation Plan and traceable to the Risk Management File	NHSP-10.3
CB-2	Consistent and reproducible triage across users, services and channels	Disposition agreement at or above the acceptance criterion for equivalent presented evidence	NHSP-10.3
CB-3	Appropriate direction of lower-acuity presentations, reducing avoidable attendance without increasing undertriage	Overtriage rate at or below the acceptance criterion, assessed jointly with CB-1 and not in isolation	NHSP-10.3
CB-4	Transparent triage reasoning at the point of assessment, through the disposition confidence value	Calibration of the confidence value within the acceptance criterion, with over-reliance and under-reliance evidenced as distinct failure modes	NHSP-10.3, usability engineering file

The declared intended purpose includes identifying presentations that may require an emergency or urgent response. That is a performance claim, made deliberately because it is what triage is for, and it cannot coexist with a disclaimer of sensitivity. Essential Requirement 3 requires the device to achieve the performance intended by the manufacturer, and Annex X requires clinical data demonstrating it. CB-

1 and CB-3 therefore carry acceptance criteria in the Clinical Evaluation Plan, evidence in the Clinical Evaluation Report, and traceability to the Risk Management File.

2.4 Target Patient Population

2.4.1 Primary Population

Age range. All patient ages: neonates, paediatric patients, adults and elderly patients. No age restriction applies; the device includes age-specific clinical pathways validated for each age group, and this includes members of the public of any age using the self-service channel where consent and access arrangements permit.

Sex. No restriction. The device is intended for patients of all sexes.

Clinical condition and disease stage or severity. As set out at 2.2.2: any symptom, health concern or request for health-related advice, across all body systems, from immediately life-threatening presentations to lower-acuity and non-urgent care needs, with no restriction to a specific diagnostic code.

Other relevant characteristics. The device may be used for individuals with co-morbidities and relevant clinical factors identified during assessment. Complex past medical history or co-morbidities may require escalation to a clinician or clinical advisor in accordance with service procedure; not all complex presentations will be identified as requiring escalation at the outset, and clinical complexity may emerge during the assessment.

Risk consideration. Given that the device is used to assess individuals across a broad spectrum of acuity, including potentially life-threatening presentations, triage outputs must be used appropriately within defined protocols. Inappropriate triage decisions, whether due to misuse, incorrect input or other factors, may result in delayed access to care and potential deterioration of a patient's condition.

2.4.2 Sub-populations and Considerations

Paediatric patients (under 18). Indicated. The device includes dedicated, clinically validated paediatric assessment pathways; users, and individuals in the self-service channel reporting for a child, are guided through age-appropriate clinical questions.

Pregnant patients. Indicated. Specific clinical logic is included to identify and appropriately manage presentations in pregnant patients.

Patients with complex co-morbidities. Not treated as a predefined sub-population. They may be assessed either through the standard structured triage pathway based on the presenting symptom, or, where appropriate, through a complex-call past medical history route subject to NHS Pathways system training principles. In all cases the assessment begins with the emergency screen, ruling out the need for an emergency response before progression to symptom-based pathways. Evaluating evidence across multiple presenting symptoms at once, rather than working a single fixed pathway to its end before considering another, is a characteristic of the adaptive model that is relevant to this population, though it is not a substitute for the complex-call route where service procedure requires one.

Non-English-speaking patients. The device presents in English. In assisted channels, organisations deploying the device are responsible for ensuring appropriate language support, such as interpreter services, is available; this is a deployment-level risk control recorded in the Hazard Log. In the self-service channel there is no advisor present to arrange interpretation, which makes language support a materially harder risk to control than in the assisted channel and is recorded as a distinct hazard rather than inherited unchanged from Core Pathways.

Patients unable to communicate symptoms directly, and proxy or third-party reporting. In assisted channels the device supports third-party reporting: a caller, such as a carer, bystander or healthcare professional, may report symptoms on behalf of the patient, and clinical pathways include logic to account for this. Whether the self-service channel supports or excludes third-party reporting is not yet declared. Proxy reporting is a material accuracy risk for a device that relies entirely on self-report, and leaving it undeclared for the self-service channel leaves that risk unowned. Open item at 2.12.

2.4.3 Excluded Populations

Individuals accessing the device other than through an approved host product or an approved digital self-service channel are excluded, consistent with the accreditation position at 2.2.7.

This device's population is not excluded from lay, patient-facing use in the way CP-2.4 and CP-2.6 exclude it for Core Pathways: the declared self-service channel is a deliberate extension of scope to members of the public, and is the single most consequential difference between this device's population and the Core Pathways baseline. It is stated here directly rather than left to be inferred, because it is the population change most likely to be missed by a reviewer working from the Core Pathways precedent. The user population that is excluded, and the contraindicated users and contexts, are stated in full at 2.5 Contraindications and 2.6.5 User Contraindications.

2.4.4 Population Scale

For context, CP-2.4 records that approximately 28 million calls were triaged through NHS 111 and NHS 999 in 2024. That figure describes the Core Pathways platform as a whole and is not a deployment volume for this device: the device operates within the same NHS 111, NHS 999 and Clinical Assessment Service workflows, extended to the self-service channel, but only within whatever trial or rollout configuration is current. No device-specific population scale or deployment volume figure is asserted in this document. Precise deployment volumes for the current configuration are to be documented in the Clinical Evaluation Plan and monitored under the Post-Market Surveillance Plan (NHSP-10.3, NHSP-10.4) once that configuration is fixed, consistent with how Core Pathways treats the same figures.

2.5 Contraindications

2.5.1 Out-of-Scope Uses

The following uses are explicitly outside the intended purpose of the device and must not occur:

- Use to establish, confirm or exclude a diagnosis.
- Use to determine treatment, to advise on the use of any medicine, or to manage ongoing clinical care, beyond the fixed first-aid, immediate-action and safety-netting content attached to a disposition.
- Use as the sole basis for a clinical decision where the user knows the presenting information to be incomplete or unreliable.
- Use by personnel who have not completed mandatory NHS Pathways training, in channels where that training is a declared risk control.
- Use by a non-clinical service advisor outside the pathway subset they are authorised for, consistent with CP-2.5 and CP-2.6.
- Use outside the defined NHS service workflows and operational environments for which the device is configured.
- Use outside England, the current deployment scope stated at 2.7.6. This is a narrower limit than the Great Britain regulatory scope at 2.9, and the two must not be conflated: the device is not intended to be used outside Great Britain at all, and within Great Britain it is not currently deployed outside England.
- Reliance on the device's use of the individual's clinical record, previous triage outcomes, diagnoses or medications: the device assesses only the information reported during a single contact episode and has no access to any of these.
- Reliance on the device to interpret free text or to act on unstructured clinical history: it operates exclusively on structured inputs received through its interface, and it does not capture a physiological signal from any patient-reported value, only the reading the individual gives it.

- Reliance on the device to detect a mis-mapped presenting complaint: where a host product maps a description onto the wrong clinical concept, the device will reason correctly on the wrong evidence.

Consequence of misinterpretation. Misinterpretation of triage outputs as definitive clinical diagnoses or treatment instructions constitutes misuse and may lead to inappropriate patient management. The accuracy of the assessment depends entirely on the accuracy and completeness of what is reported; the device has no independent means of validating the information it receives.

2.5.2 Contraindications

- Use by personnel who have not completed mandatory NHS Pathways training, in channels where that training is a declared risk control.
- Use by a non-clinical service advisor outside the pathway subset they are authorised for.
- Use outside the defined NHS service workflows and operational environments for which the device is configured.
- Use to establish, confirm or exclude a diagnosis.
- Use to determine treatment, to advise on the use of any medicine, or to manage ongoing clinical care.
- Use as the sole basis for a clinical decision where the user knows the presenting information to be incomplete or unreliable.

2.5.3 Excluded Populations and User Contraindications

Excluded populations are stated at 2.4.3, and the contraindicated user types and contexts, including the position on untrained personnel, service advisors operating outside their authorised pathway scope and personnel accessing the device via non-approved host systems, are stated in full at 2.6.5. They are cross-referenced here rather than restated, consistent with the single-document approach explained at 2.1.1, and are together cross-referenced to the Risk Management Plan [NHS-Pathways-TF-07-001] and the Instructions for Use [NHS-Pathways-TF-09-002].

2.6 Intended Users

2.6.1 Primary Users

Direct users and determined populations are stated separately. Conflating them is how a usability obligation gets missed.

User	Description	Requirements
Trained non-clinical health advisor	Direct user in assisted channels. Operates the device within service protocol and acts on the disposition without routine clinical review of each output.	Mandatory NHS Pathways training and competency. This is a declared risk control, recorded here as an assumption of the intended purpose, and it must not be silently dropped when scope widens.
Non-clinical service advisor	Direct user in assisted channels, authorised for a restricted subset of pathways only, for example dental presentations or repeat prescription enquiries.	Mandatory NHS Pathways training scoped to the authorised pathway subset. Operating pathways outside that scope is a contraindicated use, stated at 2.5.2 and 2.6.5.
Lay, self-directed user	Direct user in approved self-service channels, and a determined population throughout. No training and no clinical knowledge assumed.	No training required, and none may be assumed. The device authors the question text and answer options that a host product renders verbatim to this user, so the BS EN 62366-1 obligation for that content sits with the device: summative evaluation for the lay

		group, a defined reading level, accessibility conformance and comprehension evidence. Expanded at 2.6.3.
Registered clinician	Direct user within a Clinical Assessment Service.	Clinical registration and service competency. Anchoring on a displayed disposition or confidence value is an evidenced failure mode and requires human factors evidence for this group. Introducing this group creates a second intended purpose within the declared statement and requires its own risk analysis under MD-QMS-SOP-D002.
Clinical administrator	Direct user of the clinical administration interface.	A defined competency for the role, role-based access control and a complete audit trail with attribution and reversion. The interface carries its own BS EN 62366-1 usability treatment; it is not covered by the usability work on the runtime interface, and its use errors have wider consequences. Misconfiguration here affects every subsequent assessment rather than one, which places it above the runtime interface in the risk analysis rather than below it.

2.6.2 Secondary and Tertiary Users

Secondary users. Clinicians and services receiving an individual whose care has been routed by the device, including receiving clinicians in CAS, ambulance services, emergency departments, urgent treatment centres and primary care. They do not operate the device. They receive the disposition and its associated parameters through the host product and the clinical record. No training is required.

Tertiary users. Those with access to the device or its records who do not influence its clinical output: IT and administrative staff, auditing clinicians, the post-market surveillance team, software maintenance staff, and coroners and legal staff.

Proxy and third-party reporting is a population characteristic rather than a user category, and is addressed at 2.4.2.

2.6.3 Lay Users and Self-Directed Use

Core Pathways records, in CP-2.6, that NHS Pathways is not intended for use by lay persons and must only be operated by trained non-clinical health advisors and clinicians deployed within approved NHS urgent and emergency care services; it treats patient-facing self-triage applications as separately classified and regulated devices, explicitly excluded from its own scope.

This device departs from that position deliberately. The declared self-service channel makes the lay, self-directed user a primary user and a determined population at once, not a secondary or excluded one. No training and no clinical knowledge may be assumed of this group. In consequence, the device itself authors and carries content, question wording and answer options, that in the Core Pathways model is mediated by a trained advisor before it reaches anyone. That shift is what drives the heightened usability obligations at 2.6.1, the negative indications at 2.10 being rendered to this user directly rather than only documented internally, and the amplified language-support hazard at 2.4.2.

2.6.4 Training and Competency Requirements

Non-clinical health advisors, service advisors and clinical administrators. Mandatory NHS Pathways training and formal competency assessment before independent use, delivered through the NHS Pathways approved training framework. Service advisors receive a subset of, or extension to, this training restricted to the pathways they are authorised to operate. Refresher training is required at each significant clinical content or model update and at minimum annually, tracked against the change management position at 2.2.6, and training records are held by the employing organisation.

Registered clinicians. Current professional registration with an appropriate body (NMC, GMC, HCPC or equivalent) and competency in urgent and emergency care assessment, aligned to NHS Pathways clinical governance standards, with refresher training as required by clinical content updates and professional registration requirements.

Lay, self-directed users. No training is required, and none may be assumed. In place of training, the device's obligation is discharged through usability evidence: summative evaluation for the lay population, a defined reading level, accessibility conformance and documented comprehension of the question wording, answer options and negative indications the device presents verbatim, held in the usability engineering file.

2.6.5 User Contraindications

Use of the device is contraindicated for the following user types and contexts, cross-referenced to the Risk Management Plan [NHS-Pathways-TF-07-001] and Instructions for Use [NHS-Pathways-TF-09-002]:

- Personnel who have not completed the mandatory NHS Pathways training programme and competency assessment for their role.
- Non-clinical health advisors operating without access to escalation pathways.
- Non-clinical service advisors operating pathways outside their authorised pathway scope.
- Personnel accessing the device via non-approved host systems or outside the NHS Pathways accreditation framework.
- Any user, lay or professional, acting on the device's output as a diagnosis or as a treatment instruction, per the negative indications at 2.10.

2.7 Intended Use Environment

This section adapts the environment characteristics documented in CP-2.7 for the declared device, and extends them for the addition of the approved self-service channel, which CP-2.7 does not address because Core Pathways excludes patient-facing use.

2.7.1 Clinical Setting(s)

NHS 999 and NHS 111 call handling. Used by trained non-clinical health advisors and clinicians to triage calls across the full spectrum of acuity, from life-threatening conditions requiring an emergency response through to lower-acuity presentations directed to home or self-care. NHS 999 is used to determine appropriate emergency response; NHS 111 triages urgent, non-urgent and life-threatening presentations and recommends appropriate care pathways.

Clinical Assessment Services. Used by clinicians to conduct remote clinical assessments and recommend care pathways for patients referred from 111 or 999 services.

Approved digital self-service channels. Used directly by members of the public, without a health advisor present, within the boundary set at 2.6.3.

Setting controls and exclusions. All assisted-channel deployment settings are controlled healthcare environments with approved host systems, clinical governance structures and trained personnel. The device is not intended for use in acute hospital inpatient, outpatient, primary care (GP surgery), community or domiciliary, or screening programme settings, in either channel.

2.7.2 Physical Operating Environment

Assisted channels. Operated from approved operational environments within NHS call centre settings and Clinical Assessment Service facilities, from fixed workstations and, where permitted under the NHS Pathways licence, approved remote working locations. Homeworking is permitted within defined licence conditions and has been confirmed as active practice with current providers.

Self-service channel. Occurs wherever the individual happens to be, unsupervised, on their own device, with no control over conditions at all. This is a materially different physical environment from the controlled, professional healthcare setting the device otherwise assumes, and it is the environment against which the self-service usability evidence at 2.6.4 is evaluated.

Environment controls and exclusions. The assisted-channel physical environment is a controlled, professional healthcare operational setting; the device is not intended for use there on personal mobile devices, consumer tablets, or in other uncontrolled environments. A stable network connection is required for operation in both channels (see 2.7.3). Assisted-channel deployment environments are subject to NHS Pathways host system accreditation requirements.

2.7.3 Technical Infrastructure

The device is deployed as a software service integrated into approved host products, accessed over a defined interface. It does not persist patient-identifiable data beyond session-level processing; data handling is governed by the host product and the NHS data governance framework.

Technical infrastructure requirements, including supported host environments and minimum specifications, are defined by the NHS Pathways host system accreditation framework and the interface specification. Consistent with CP-2.7, assisted-channel host products typically operate on Windows infrastructure within NHS call centre environments; the self-service channel is accessed through a web-based client and carries no equivalent host operating system assumption. Availability and latency requirements are specified in the interface specification, and degraded or unavailable service is a hazard treated in the Risk Management File, because a triage service that fails silently in a 999 workflow is a clinical event and not only an operational one.

The probabilistic inference engine adds a model-serving requirement beyond the rule-based Core Pathways infrastructure CP-2.7 describes: versioned model artefacts are deployed and served alongside each clinical content release, and rollback to a prior model version follows the same change control route as a content rollback under MD-QMS-SOP-D007.

2.7.4 Interoperability

Approved host products integrate the device through the defined interface, presenting its content to users and running its output within the host's own runtime. Host products are not classified as medical devices in their own right but are subject to NHS Pathways accreditation.

NHS Spine integration, where patient demographic or clinical record access is used, is indirect: it is managed at the host system level, consistent with CP-2.7, and is not an interface of the device itself.

The Directory of Services is not an interoperability interface of the device: mapping of triage dispositions to services is performed downstream by the DoS and provider processes, outside the device boundary, consistent with 2.2.1 and 2.2.4.

The device does not directly interoperate with medical devices such as diagnostic or monitoring equipment, and does not perform direct device telemetry capture; it receives only the values an individual has read from an instrument and reported themselves, as set out at 2.2.3.

2.7.5 Cybersecurity Environment

Assisted channels. Deployed within NHS Health and Social Care Network infrastructure or equivalent secure NHS network infrastructure, not directly internet-facing; access restricted to NHS-connected networks and accredited host systems. User authentication is managed at the host system level.

Self-service channel. Necessarily internet-facing, which is a different network security model from the HSCN-based assisted channel and is not addressed by CP-2.7. The authentication and access-control model for this channel, including whether and how an anonymous or lightly authenticated user is admitted, is not yet declared. Open item at 2.12.

Common controls. Patient assessment data is classified as Special Category data under the UK GDPR; data-minimisation principles apply. Data is encrypted in transit using TLS 1.2 minimum, with data-at-rest requirements specified in the NHS Pathways Technical Specification. The full cybersecurity risk assessment is in the Cybersecurity Risk Management Plan [NHS-Pathways-TF-07-002], cross-referenced to the Risk Management Plan [NHS-Pathways-TF-07-001] where risks carry patient-safety implications. The device is subject to the NHS Data Security and Protection Toolkit and NCSC Cyber Essentials requirements as applicable to NHS-hosted services, and to BS EN IEC 80001-1 and MDCG 2019-16 as adopted in the Quality Manual.

2.7.6 Deployment Geography and Jurisdiction

Regulatory scope. UK Medical Devices Regulations 2002 (SI 2002/618, as amended), applicable across Great Britain.

Current deployment. Consistent with CP-2.7, current deployment is exclusively within England, supporting NHS urgent and emergency care services. This is narrower than the Great Britain regulatory scope: the device is not currently deployed in Scotland, Wales, Northern Ireland, or outside the United Kingdom. Any future expansion of deployment geography requires review and update of this document and of the associated regulatory documentation before it takes effect, not after.

2.8 Foreseeable Misuse

Cross-reference: the mitigations below are cross-referenced to the NHS Pathways hazard log, following the same category structure CP-2.8 uses.

Category	Foreseeable misuse	Mitigation
Patient safety	Indication misuse: the device is used to answer a question it does not answer, such as what condition an individual has, or is used outside the NHS service workflows for which it is configured	Labelling and instructions for use; the negative indications at 2.10 rendered in the user-facing product; user training in assisted channels; Provider Licence and Licence to Embed terms
Patient safety	Knowledge-based misuse: a user treats the disposition confidence value as a probability of a condition, or as a measure of how serious the presentation is	Labelling; user-facing explanation of what the value means and does not mean, validated for the population that reads it; summative usability evaluation for every group that sees the value
Patient safety	Over-reliance: a user follows a disposition they have reason to doubt, or does not escalate where service criteria are met. Anchoring on a displayed disposition or confidence value is an evidenced failure mode.	Over-reliance and under-reliance treated as distinct failure modes in the risk analysis; service escalation protocols outside the device; human factors evidence for each user group
Patient safety	Under-reliance: a user overrides an urgent disposition on the basis of impression rather than protocol	Training and competency in assisted channels; audit of override patterns under NHSP-10.4
Patient safety	Inaccurate reporting: the individual, or a proxy, reports symptoms inaccurately or incompletely. The device cannot detect this.	Structured question sets with controlled answer options; hierarchical ruling-out of life-threatening presentations first; safety-netting and worsening advice on every disposition; proxy reporting to be declared in or out of scope for the self-service channel, per 2.4.2
Patient safety	Use by an untrained user in a channel where training is a declared risk control	Access control through the host product; training completion as a condition of access; the training assumption stated at 2.6.4 so it cannot be dropped without a change

		assessment
Patient safety	Mis-mapped presenting complaint, where free-text extraction outside the device returns the wrong clinical concept	Interface specification states the device's assumption about received concepts; residual risk carried explicitly per 2.2.7; confirmation step in the host product where adopted
Patient safety	Reported physiological value from an unvalidated or misread instrument	Recorded as a hazard; subgroup performance analysis addressing known instrument bias; pathway-level constraint on which values are accepted
Patient safety	Navigation and functionality-use error: given the scale and flow of triage, human error in navigating the assessment or in using available functionality is a known and foreseeable misuse case, in both channels T	raining, competency assessment and instructions for use in assisted channels; usability evidence and interface design for the self-service channel per 2.6.3 and 2.6.4
Patient safety	Use outside intended settings: deployment or access in a setting outside those declared at 2.7.1, for example a primary care or hospital ward setting whose clinical context and governance are not aligned to this i	ntended purpose Accreditation restricted to approved host products and approved self-service channels per 2.2.7; setting controls and exclusions stated at 2.7.1
Patient safety	Self-service channel accessed by, or on behalf of, an individual for whom the channel's authentication or safeguarding position has not yet been settled, including a proxy reporting for someone else, per the open	items at 2.12 Interim reliance on the negative indications at 2.10 and the usability evidence at 2.6.4; authentication and safeguarding position to be settled and this row updated once OI-1 and OI-5 are closed
Configuration	Misconfiguration through the clinical administration interface: an incorrect threshold, content version or disposition mapping released to production. Affects every subsequent assessment rather than one.	Defined competency for the role; role-based access control; complete audit trail with attribution and reversion; change control under MD-QMS-SOP-D007 and clinical safety authorisation under MD-QMS-WI-D002
Service	Continued use of the device where the service is degraded, unavailable or returning stale content	Availability and latency requirements in the interface specification; host product fallback behaviour specified; degraded service treated as a clinical hazard in the Risk Management File
Cyber-security	Unauthorised alteration of clinical content, thresholds or model parameters; interception or alteration of assessment data in transit; unauthorised access via the internet-facing self-service channel	Access control and audit; secure interface; NHS Cyber Essentials Plus; BS EN IEC 80001-1 and MDCG 2019-16 as adopted in the Quality Manual; self-service channel authentication model to be settled per the open item at 2.12
Individual rights	Assessment data used for purposes other than those for which it was provided, including in post-market surveillance	Data Protection Impact Assessment; NHS data governance framework; data handling responsibilities allocated between the device, the host product and the provider

2.9 Classification Determination

This section and 2.10 extend beyond the Core Pathways scheme: they are specific to this device's own conformity route and are not mirrored in CP-2.1 to CP-2.8.

2.9.1 Qualification

The device acts on data relating to an identifiable individual, for a medical purpose within the meaning of regulation 2(1), and creates new information from that data rather than presenting, storing or transmitting what it received. It is standalone software qualifying as a medical device. Annex IX Section I paragraph 1.4 provides that stand alone software is considered to be an active medical device. Classification is given effect in Great Britain by regulation 7(1) of SI 2002/618.

2.9.2 Rule-by-rule Determination under Annex IX

Rule	Application E	ngaged
Rule 9, active therapeutic devices administering or exchanging energy	The device administers no energy and provides no therapy. It does not control or monitor an active therapeutic device.	No
Rule 10, active devices for diagnosis	Limb 1, energy absorbed by the body other than illumination in the visible spectrum: not met. Limb 2, imaging the in vivo distribution of radiopharmaceuticals: not met. Limb 3, direct diagnosis or monitoring of vital physiological processes: on monitoring, the device monitors no vital physiological process, because it receives reported observations and captures no physiological signal. On direct diagnosis, the device does not allow direct diagnosis on the declared output set. See 2.9.3. Rule 10 also places a device in Class IIb where it is specifically intended for monitoring vital physiological parameters whose variation could result in immediate danger to the patient, and where it emits ionising radiation for interventional radiology. Neither is engaged. That Class IIb	limb is the reassessment trigger recorded at boundary condition 2 and OI-6 (2.11, 2.12). No
Rule 11, active devices administering or removing medicines, body liquids or other substances	Not engaged. Class IIa, or Class IIb where administration is performed in a potentially hazardous way. Note that this is Annex IX Rule 11, a different provision from EU MDR 2017/745 Rule 11 governing software; the two share a number and nothing else.	No
Implementing rule 2.3, software driving or influencing the use of a device	The device does not drive or influence the use of another active medical device. Carried as boundary condition 8 at 2.11.	No
Rules 13 to 18, special rules	Devices incorporating a medicinal substance; contraception and prevention of sexually transmitted disease; disinfection of devices and of contact lenses; recording of X-ray diagnostic images; devices using non-viable animal tissue or derivatives; blood bags. Each considered. The device is software only, incorporates no substance, no tissue and no radiation source. None is engaged.	No
Rule 12, all other	The fallback, available where no other rule is engaged.	**Yes**

active devices are Class I	Under Annex IX Section II implementing rule 2.5, where several rules apply the strictest rules resulting in the higher classification apply. None stricter applies here.	

Recorded classification: Class I. Conformity is self-declared under Annex VII of Directive 93/42/EEC. UKCA marking, a Declaration of Conformity and registration with the MHRA are required before the device is placed on the GB market, and the manufacturer carries full responsibility for the technical documentation, the Essential Requirements and post-market surveillance.

2.9.3 Rule 10 Direct Diagnosis, and the MHRA Symptom Checker Guidance

MHRA guidance v1.10f Appendix 1 is the most directly applicable UK source on this product type, and it is direct on the outcome: symptom checker devices will be Class I unless considered to allow direct diagnosis, in which case they will be Class IIa.

Appendix 1's operative tests are whether the software provides the diagnosis of the disease or condition by itself, whether it provides decisive information for making a diagnosis, and whether claims are made that it can perform as, or support the function of, a clinician in performing diagnostic tasks. On the declared output set none of the three is met. The output vocabulary is the NHS Pathways disposition set. No output names, ranks or attaches a likelihood to a clinical condition, and the confidence value expresses confidence in a route to care rather than the likelihood of a condition.

Appendix 1 also lists indicative words and phrases in the context of allowing direct diagnosis, and "triage" is among them. It is the declared function of this device, so the point is addressed here rather than avoided. The list is indicative, not determinative, and the operative tests are the three above. Appendix 1's examples are consistent with that reading, and the distinction they draw is worth stating precisely. Software intended to output a subset of medical conditions matching the input symptoms, and software offering filters by red flag, severity or probability of a match, are listed as software likely to qualify as a device. Software that only signposts the user to suitable care is listed as unlikely to. Those examples go to qualification rather than to the direct diagnosis test, and qualification is conceded here in any event at 2.9.1. On the classification question this device does neither of the first two things: it returns one disposition, a confidence in that disposition, and the parameters for reaching care.

Appendix 1 adds that for devices intended to be used by lay users, the provision of an indicative diagnosis may be enough to imply that the device is allowing direct diagnosis. That sentence is engaged by the declared lay self-service channel at 2.6.3 and is answered on the output set rather than avoided. Nothing the device returns to a lay user is an indicative diagnosis. The output is a disposition, its timeframe and skill set, fixed safety-netting content, and a confidence in the disposition. No output names a condition, no output ranks conditions, and the confidence value expresses confidence in a route to care. That is stated to the lay user as a negative indication at 2.10 and is evidenced for comprehension in the usability engineering file.

Consistency anchor, and its limit. NHS 111 online is published by NHS England as a Class I medical device carrying the UKCA mark under the UK MDR 2002. It uses the same NHS Pathways clinical content, in the same territory, and returns dispositions from the same set. The anchor establishes two things: that a triage device returning an NHS Pathways disposition to the public is Class I in NHS England's own registered position, and that the reasoning method is not a classification criterion.

2.9.4 IEC 62304 Software Safety Classification

Class C. Undertriage of a time-critical presentation can lead to death, the device does not require routine clinical review of each output, and the emergency screen is the control on which the safety argument rests. Severity is assessed before external risk control measures are credited, which is NHS England policy under MD-QMS-WI-D002 rather than a requirement of the standard, and is adopted deliberately. Decomposition inherits the parent class, so every component of the device is Class C unless separation is demonstrated. Under MD-QMS-WI-D002 a decomposition argument requires segregation on different machines or processors. That, like the assessment of severity before external risk controls, is NHS England policy adopted deliberately rather than a requirement of IEC 62304, which requires only that segregation be documented and demonstrably effective and offers separate processors as one example.

IEC 62304 class is independent of device class. This device is Class I under Annex IX and Class C under IEC 62304 at the same time. Lifecycle rigour is set by safety; the conformity route is set by classification. Conflating the two is the most common way a Class I position gets mistaken for a light-touch one.

The consequence of holding Class I with Class C software is that no third party reviews the lifecycle. That is a foreseeable question and the answer is not the classification. It is that clinical safety authorisation under MD-QMS-WI-D002 is a mandatory release gate for every Class B and C item, that the clinical safety case is independently reviewed by the Clinical Safety Officer under DCB0129, and that a Class I self-declaration reduces no Essential Requirement and no Annex X clinical evaluation obligation.

2.10 Negative Indications and Labelling

The following statements appear in the instructions for use and in the user-facing product, and they form part of the declared intended purpose rather than commentary on it.

1. This device does not provide a diagnosis. It does not establish, confirm or exclude any clinical condition.
2. This device does not produce a list of possible conditions and does not indicate the likelihood of any condition.
3. The confidence value shown expresses confidence in the recommended route to care. It is not a probability that any condition is present, and it is not a measure of how serious a presentation is.
4. This device recommends a route to care. It does not book, allocate or initiate any service, appointment or response.
5. This device does not manage treatment and does not advise on the use of any medicine.
6. This device assesses only the information reported during this contact. It does not use the individual's medical record or the outcome of any previous contact.

A disclaimer does not override a function. Where the declared purpose and the observable behaviour of the software diverge, the behaviour governs. Each of these six statements is therefore a design constraint that must be true of the software, verified before release rather than asserted at release.

Marketing, service communications and user-facing content are reviewed against the registered intended purpose. Under the misleading claims provision proposed in the draft 2026 Regulations, promoting a use outside the scope of the conformity assessment becomes a prohibited claim in its own right.

2.11 Boundary Conditions

Referenced from 2.2.6, 2.2.7 and 2.9.2 but absent from the previous draft of this document. Reconstructed here from NHSP-CIR against the record held in project memory, and flagged for verification against NHSP-CIR §10 before this document is approved.

These are the conditions that hold the Class I classification. Crossing 1, 2 or 3 moves the device to Class IIa on its own. Condition 4 does the same where the assurance given carries condition-level information. Conditions 5 to 10 do not move the UK class on that ground alone, but they move the IEC 62304 class, the residual risk position, and the EU position, and are tracked for that reason.

ID	Boundary condition	Consequence if crossed
1	No condition-level information is generated, retained, exposed or used for service selection.	Class IIa
2	No physiological signal capture or image interpretation.	Class IIa (Rule 10 monitoring or

		imaging limb)
3	No condition-level or severity value is generated, and no host threshold routing on the confidence value without that routing being declared.	Class IIa
4	No clinical assurance is given to an individual, in the absolute form that the unreviewed lay channel requires.	Class IIa where the assurance carries condition-level information
5	No autonomous clinical action is taken by the device.	IEC 62304 class, residual risk and EU position
6	Single contact episode only; no carried-over evidence between contacts.	IEC 62304 class, residual risk and EU position
7	Adaptive behaviour is bounded, and no question is selected to discriminate between named conditions.	IEC 62304 class, residual risk and EU position
8	No substances, energy or control of another device.	IEC 62304 class, residual risk and EU position
9	Advice content is fixed and impersonal.	IEC 62304 class, residual risk and EU position
10	Clinical parameters are held only inside the device boundary.	IEC 62304 class, residual risk and EU position
11	The clinical administration interface is not a care interface.	IEC 62304 class, residual risk and EU position

2.12 Open Items

Referenced from 2.2.6, 2.2.7, 2.3.1, 2.4.2, 2.7.5 and 2.9.2. Each must be closed, or explicitly accepted with reasoning recorded, before this statement is approved.

This register is a reconstruction, not a recovery of the original. The previous draft's inline reference to "open item 9" implies the original open items register held at least nine entries; this reconstruction has six, built only from what the surviving text of v0.3 pointed to plus what the v0.4 restructuring itself surfaced. The remainder cannot be recovered from this document and must be sourced from whoever holds the version of this statement, or of PT-2.1-A, in which the original register was still present, before this section is treated as complete.

ID	Open item	Raised at
OI-1	Proxy and third-party reporting in the self-service channel is not yet declared in or out of scope.	2.4.2
OI-2	The hazard reference for the stopping threshold value is open, pending assignment in the Risk Management File.	2.2.6
OI-3	The regulatory status of the host product, accessory, IT environment or component, is not yet determined and is a prerequisite to approval.	2.2.7
OI-4	No literature search confirming the absence of an equivalent SaMD has yet been completed for the probabilistic components.	2.3.1
OI-5	The authentication and access-control model for the internet-facing self-service channel is not yet declared.	2.7.5
OI-	Whether the device is specifically intended for	ngaging the Rule 10 Class IIb monitoring limb,

6	monitoring vital physiological parameters whose variation could result in immediate danger, e	has not been separately reassessed against the current patient-reported physiological value pathways. 2.9.2

Document references

Reference	Title
UK MDR 2002	The Medical Devices Regulations 2002, SI 2002/618, as amended. Regulation 7(1) gives effect to the Annex IX classification criteria.
93/42/EEC	Council Directive 93/42/EEC concerning medical devices, Annex IX classification criteria, as given effect in Great Britain by the UK MDR 2002
MHRA software guidance	MHRA, Guidance: Medical device stand-alone software including apps (including IVDMDs), v1.10f, July 2023, in particular Appendix 1 Symptom checkers
MEDDEV 2.1/6	Guidelines on the qualification and classification of stand alone software used in healthcare within the regulatory framework of medical devices
MEDDEV 2.4/1 Rev 9	Classification of medical devices, June 2010
EU MDR 2017/745	Regulation (EU) 2017/745 on medical devices, Annex VIII Rule 11. Recorded for Northern Ireland and forward planning.
MDCG 2019-11 Rev 1	Guidance on qualification and classification of software in Regulation (EU) 2017/745 and Regulation (EU) 2017/746, revision 1, June 2025
IMDRF/SaMD WG/N12 FINAL:2014	Software as a Medical Device: possible framework for risk categorization and corresponding considerations
IMDRF/SaMD WG/N81 FINAL:2025	Characterization considerations for medical device software and software-specific risk, 27 January 2025
BS EN 62366-1:2015+A1:2020	Medical devices, Part 1: Application of usability engineering to medical devices
PD IEC/TR 62366-2:2016	Medical devices, Part 2: Guidance on the application of usability engineering to medical devices
BS EN ISO 14971:2019+A11:2021	Medical devices, Application of risk management to medical devices
BS/AAMI 34971:2023	Application of BS EN ISO 14971 to machine learning in artificial intelligence, Guide
IEC 62304:2006+A1:2015	Medical device software, Software life cycle processes
BS EN 82304-1:2017	Health software, Part 1: General requirements for product safety
BS EN ISO 14155:2020+A11:2024	Clinical investigation of medical devices for human subjects, Good clinical practice
MEDDEV 2.7/1 Rev 4	Clinical evaluation: a guide for manufacturers and notified bodies under directives 93/42/EEC and 90/385/EEC, June 2016. Source of the clinical benefit definition used at 2.3.2.
DCB0129	Clinical risk management: its application in the manufacture of health IT systems
DCB0160	Clinical risk management: its application in the deployment and use of health IT systems
Draft 2026 Regulations	Draft Medical Devices (Amendment) Regulations 2026, published via the WTO notification portal on 8 May 2026. MHRA call for evidence closed 19 June 2026. Adoption anticipated December 2026 with provisions expected to come into force from June 2027, on the MHRA’s published indicative timeline. Applied for forward planning only. Not in force, and nothing in this

	document relies on it.